TOGAF 10 · PRELIMINARY PHASE

Executive Brief: Barangay San Antonio Health Center

Pasig City Health Department · UA&P Solution Architecture

October 2, 2026 · Primary interviewee: Mafe B. Abong, RM, Rural Health Midwife

~80
patients per week (~340 per month)

16
on-site staff, no records clerk

10 min
to find a misfiled chart, vs ~5 s in the EMR

7
program reports compiled by 1 midwife each month

1 Enterprise Boundary & Problem Authenticity

Q1.1 Enterprise Scope & Landscape Baseline

Barangay San Antonio Health Center is a single-site primary-care facility under the Pasig City Health Department. Its 16 on-site staff serve about 80 patients a week. Field work also runs from neighborhood stations and door-to-door visits during drives.

Area	Baseline
Staff (16)	1 Medical Officer IV, 1 dentist, 1 rural health midwife, 1 rural health nurse, 1 nutritionist-dietitian, 1 Barangay Nutrition Scholar, 1 IT officer/medical encoder, 4 Pasig Health Aides, 3 BHWs, 1 security, 1 utility
Services	Consultation, prenatal (Safe Motherhood), family planning, sick infant, infant/adult immunization (NIP, MR-SIA), TB, animal bite, dental, wound care/minor surgery, OPT Plus nutrition, dengue prevention, PhilHealth YAKAP/GAMOT medicines
Digital	Pasig Health Monitor (city-owned EMR shared with other health centers and city hospitals); staff group chats for supplies and operations
Paper	Individual Treatment Record (ITR), per-visit Patient Chart with consent, vaccine cards, PhilHealth GAMOT logbook (UPSC, QR code), Records Storage/Retention/Disposal logbook, Visitors logbook
Hardware	4 laptops + 1 printer issued by the City; 4 workstations + 1 printer issued by the Barangay
Limitations	No on-site records clerk (the Records Officer sits at city level); EMR lookup needs stable internet; only the City Health Department can change the EMR; no end-of-life or license cliff reported

Q1.2 Business Catalyst

Patient service and monthly compliance reporting both depend on a **split paper-and-EMR record that no one is assigned to keep in order**. A misfiled chart takes up to 10 minutes to find, versus about 5 seconds in the EMR. Meanwhile one midwife must consolidate 7 program reports between the 30th and the 1st for City Health Office review before a Wednesday deadline. New PhilHealth YAKAP/GAMOT dispensing adds a third record to keep.

Q1.3 Primary Operational Failure Modes

Failure 1

Lost time at the records shelf

Filing is a shared task, so charts get misfiled or duplicated. Lookup rises from 2–3 minutes to up to 10 minutes, and an internet drop removes the 5-second EMR path. Lost vaccine cards force a manual archive search.

Failure 2

Single-person dependencies

One midwife compiles every monthly program report, and one physician runs consultations. When the physician is absent, patients are referred to Oranbo Health Center, Kapitolyo Health Center or Rizal Medical Center, escorted by BHWs in barangay vehicles.

2 Organizational Model for EA

Q2.1 Scope of Organizations Impacted & Governance

The architecture boundary is the health center itself: its 16 staff, its paper records and its 8 devices. **Executive Sponsor: Dr. Glynnis Lynn U. Bonilla, Medical Officer IV.**

Inside the boundary

All 16 center staff; paper records and logbooks; center-side use of the EMR.

Barangay governance

Punong Barangay Raymond Thomas U. Lising; Kagawad Rachel Rustia, Chair of the Committee on Health (barangay budget and the 4 barangay workstations).

Outside (dependencies)

Pasig City Health Department: City Health Officer Joseph R. Panaligan, MD; Chief Administrative Officer Philip Patrick C. Co, MD; Records Officer Rosa C. Santos; Secretary Officer Algernon A. Victorino. City Mayor Victor Ma. Regis N. Sotto; Pasig Health Monitor EMR; PhilHealth (YAKAP/GAMOT); DOH and National Nutrition Council; referral facilities.

Architecture Review Board

Dr. Bonilla (chair), Mafe B. Abong, RM, Nita N. Hardman, RN, Tito F. Reyes (IT officer) and Kagawad Rustia. Any change to processes or the EMR then passes a City Health Department approval gate.

Q2.2 Stakeholder Alignment & RACI Matrix

Top 3 stakeholders: **Dr. Bonilla** (authority over the center), **Mafe B. Abong** (runs reporting and maternal care) and **Kagawad Rustia** (controls barangay funding).

Activity	Responsible	Accountable	Consulted	Informed
Ratify architecture principles	Student EA team	Dr. Bonilla	Abong, Hardman, Reyes, Kag. Rustia	City Health Dept., all staff
Sign off Phase A (Architecture Vision)	Student EA team	Dr. Bonilla	Kag. Rustia, Dr. Co (City Health Dept.)	All center staff
Approve process or EMR changes	Dr. Bonilla	City Health Officer	Abong, Reyes	All center staff
Fund barangay-side tools	Kag. Rustia	Punong Barangay Lising	Dr. Bonilla	Center staff
Monthly program reporting	Abong, with health aides and BNS	Dr. Bonilla	CHO program coordinators	Kag. Rustia

Q2.3 Framework Interface Assessment

ADM work is scheduled around the center's fixed operating rhythms and external standards.

Rhythm or framework	Timing	Effect on the ADM cycle
Monthly program reports	Data 30th → 1st; CHO review before a Wednesday deadline	No workshops in the last 2 days or first week of a month
Operation Timbang Plus	January–March (children 0–59 months)	No changes rolled out; nutrition baseline captured then
Immunization drives (MR-SIA)	Periodic, 9–11 AM in the field	ADM sessions held in the afternoon
Barangay budget	Annual proposals to the Barangay Council	Phase E roadmap items timed to the proposal cycle
City Health Department approval	Any process or record-system change	Becomes the Phase G compliance gate
DOH/NNC BNS Handbook on the First 1000 Days; PhilHealth YAKAP/GAMOT rules; Data Privacy Act (RA 10173)	Continuous	Treated as fixed requirements in Phases B–C

3 Tailored Architecture Framework

Q3.1 Metamodel Tailoring Logic

26
of 36 core entities retained

3
deleted

7
merged into others

+8
health extensions → 34 in total

Decision	Count	Entities and reason
Deleted	3	Product, Contract (public services are free and not sold under contract); Course of Action (strategy is set by the City, not the center)
Merged	7	Logical and Physical Application Component → Application Component, since the estate is one EMR plus paper; Logical and Physical Technology Component → Technology Component, since there are only 8 devices; Physical Data Component → Logical Data Component; Service Quality → Measure; Information System Service → Application Component
Retained	26	Including Actor, Role, Organization Unit, Location, Business Service, Function, Process, Event, Value Stream, Capability, Data Entity, Application Component, Technology Component, Driver, Goal, Objective, Measure, Principle, Requirement, Constraint, Assumption, Control, Gap, Work Package
Added	8	Individual Treatment Record; Consultation Encounter; Consent Record; Health Program (Safe Motherhood, NIP, TB, OPT Plus and others); Monthly Program Report; GAMOT Dispensing Entry (UPSC, QR code); Referral; Record Retention & Disposal Entry

The Visitors logbook is out of scope because it holds no patient or program data.

Q3.2 Committed Architecture Deliverables

Phase	Catalogs, matrices and diagrams
A: Architecture Vision	Stakeholder Map Matrix; Value Chain diagram of a patient visit; Architecture Vision diagram
B: Business	Business Capability Map; Organization/Actor Catalog; Role/Actor Matrix; process diagrams for patient visit and record lookup, monthly program reporting, and referral
C: Data	Data Entity Catalog; Logical Data diagram (ITR, Encounter, Consent, Program Report); Data Entity/Business Function (CRUD) Matrix; Data Lifecycle diagram for retention and disposal

Phase	Catalogs, matrices and diagrams
C: Application	Application Portfolio Catalog; Application Communication Diagram (paper ↔ EMR ↔ PhilHealth ↔ City Health Dept.); Application/Function Matrix
D: Technology	Technology Portfolio Catalog (8 devices, 2 printers, City vs. Barangay owner); Environments & Locations diagram showing the internet dependency
E: Opportunities & Solutions	Gap Analysis; Architecture Roadmap timed to the barangay budget cycle; Transition Architecture views

4 Collaborative Foundations & Evidence

Q4.1 Principles Catalog

Principle 1 · One Patient, One Record	
Statement	Every patient has exactly one authoritative record, and filing rules work no matter which staff member files.
Rationale	Filing is shared and there is no on-site clerk, so charts are misfiled or duplicated; lookup rises from 2–3 to up to 10 minutes.
Implications	All 16 staff use one index: the ITR record number, linked to the EMR. Staff time is set aside to merge duplicates. First-visit intake takes longer.

Principle 2 · Use the City Record, Don't Rebuild It	
Statement	The city EMR is the system of record; any center-level tool feeds it and never creates a separate copy.
Rationale	The EMR is shared with other health centers and city hospitals, returns a record in about 5 seconds, and only the City Health Department may change it.
Implications	The center gives up building its own local database. Any offline fallback must sync back to the EMR. Every proposal passes the City Health Department gate, which is slower but keeps one version of the truth.

Principle 3 · Report Once, Count Many Times	
Statement	Each service is recorded once, at the point of care, and every monthly program report is derived from that entry.
Rationale	One midwife compiles 7 program reports in a one-day window (30th → 1st) before a Wednesday review deadline.
Implications	Encounters carry standard program tally fields. Health aides and the BNS record on the same day. Consultations gain a little structure during busy hours, and month-end rework drops.

Q4.2 Primary Data Sources & Baseline Metrics

- **Interviewee:** Mafe B. Abong, RM, Rural Health Midwife, interviewed on October 2, 2026.
- **Documents seen:** the center's organizational chart poster; blank Individual Treatment Record and Patient Chart forms; field notes on the GAMOT, Records Retention & Disposal, and Visitors logbooks; the DOH/NNC *Barangay Nutrition Scholar Handbook on the First 1000 Days*.

Verified metric	Value	Source
Patient volume	80 per week (~340 per month)	Interview
On-site staff	16	Org chart
Devices	4 City laptops + 4 Barangay workstations; 2 printers (1 each)	Interview
Chart lookup time	2–3 min filed, up to 10 min misfiled, ~5 s in the EMR	Interview
Reporting window	7 program reports; data 30th → 1st; CHO review before Wednesday	Interview
OPT Plus	January–March, children 0–59 months	Interview